

Prior Authorization Request

Health Plan

Humana Medicare Advantage -- Gold Choice PPO.

Member Information

Member: Gwendolyn Okafor-Reyes DOB: 1952-11-03 (age 73) Member ID: MBR-57731

Sex: F

Requesting Provider

Practice: Cedar Hollow Cardiovascular Group

Requested Service

Requested service: Elective inpatient admission, implantation of a transvenous implantable cardioverter defibrillator (ICD)

CPT 33249

Planned admit date: 2026-10-15

Level of care requested: INPATIENT

Made-up data for a software prototype. Not a real patient.

Cardiology office visit

CEDAR HOLLOW CARDIOVASCULAR GROUP

Office Visit 09/29/2026 | Okafor-Reyes, Gwendolyn | DOB 11/03/1952

Provider: Marcus T. Delahaye, MD

CC: f/u ischemic CM, ICD discussion

HPI: 73F with anterior STEMI 02/2023 s/p LAD stent, ischemic CM, PAF on apixaban, CKD 3a. Had exertional angina over the spring (CCS III), now resolved after staged PCI this summer. Main issue now is dyspnea - walks to mailbox and has to rest, can't do her laundry downstairs without stopping. No rest sx, no orthopnea. Denies syncope. Palpitations rare.

Vitals: BP 112/64 HR 68 irregular? -> ECG SR w/ PACs Wt 152 lb

Exam: JVP 7 cm, lungs clear, no edema.

Imaging: TTE 09/14/26 LVEF 28%, anterior/apical AK.

A/P:

1. ICM, HFrEF, NYHA III. On GDMT >3 yrs, at max tolerated doses. Refer EP for primary prevention ICD.
2. CAD - no angina post PCI. DAPT -> clopidogrel + apixaban (no ASA) per triple-therapy avoidance protocol.
3. PAF - rate controlled, HR 60s-70s on metoprolol, CHA2DS2-VASc 5, cont apixaban.
4. CKD 3a - stable.

RTC 3 mo.

M. Delahaye MD

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Transthoracic echocardiogram

Cedar Hollow Imaging Center - Echocardiography Report

Exam date: 09/14/2026 Patient: OKAFOR-REYES, GWENDOLYN Age 73 F

Rhythm: sinus

LV: dilated (LVIDd 5.9 cm). Akinetic mid-distal anterior wall, anteroseptum and apex. Remaining segments hypokinetic.

LVEF by biplane method of disks: 28 %

RV: normal size, mildly reduced fxn (TAPSE 1.6).

Valves: mild-mod MR, mild TR. Est RVSP 41 mmHg.

No apical thrombus with contrast.

Conclusion: Severe LV systolic dysfunction, LVEF 28%. Compared with 03/2025 (LVEF 30%), no significant change.

Interpreting physician: A. Rasmussen-Cole, MD

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Shared decision making note

Cedar Hollow Cardiovascular Group - Device Clinic

SDM visit for ICD Date of encounter: 10/01/2026

Patient: Gwendolyn Okafor-Reyes Present: patient, daughter (Ngozi)

Facilitator: Trina Beaulieu, PA-C

Decision aid: validated ICD option grid / decision aid (printed, plain-language version), reviewed line by line.

Discussed benefit in absolute terms (approx 7 of 100 people avoid sudden death over 5 yrs), risks: infection, lead problems, inappropriate shocks, psychological impact, deactivation options.

Patient's priorities: independence, staying out of hospital. Daughter asked about MRI compatibility - answered.

Patient chose ICD. Understands she may decline at any time.

Decision aid copy scanned to chart.

T. Beaulieu PA-C

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Laboratory results

Quest-style outpatient panel Collected: 09/25/2026

OKAFOR-REYES, G F 73y

BMP: Na 140, K 4.4, Cl 104, HCO₃ 25, BUN 24, Cr 1.3 H, eGFR 42 L, Glu 101

CBC: WBC 6.1, Hgb 11.6 L, Hct 35.2, Plt 188

Coags: PT 13.0, INR 1.1 (on apixaban, last dose prior evening)

NT-proBNP: 2,240 H

Lipids: LDL 54, HDL 48, TG 110

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Medication list

ACTIVE MEDICATIONS (printed 09/29/2026)

- metoprolol succinate 100 mg PO daily since 03/02/2023
- sacubitril-valsartan 49/51 mg PO BID since 03/02/2023 (switched from losartan)
- spironolactone 12.5 mg PO daily since 05/2023
- dapagliflozin 10 mg PO daily since 01/2024
- apixaban 5 mg PO BID since 2024 (PAF)
- clopidogrel 75 mg PO daily since 07/20/2026
- rosuvastatin 40 mg PO daily since 2023
- furosemide 40 mg PO daily since 2023
- levothyroxine 50 mcg daily long-standing

Discontinued: aspirin 81 mg (stopped 08/20/2026, on OAC + P2Y12)

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Cardiac catheterization and PCI report

CEDAR HOLLOW REGIONAL MEDICAL CENTER - CARDIAC CATH LAB

Procedure date: 07/20/2026 Patient: Okafor-Reyes, Gwendolyn MRN CH-0081174

Operator: L. Fenwick, MD Access: R radial 6F

Indication: staged elective PCI; CCS class III angina, positive stress test, known mid LCx stenosis.

Findings: LM nl. LAD - patent prior stent (2023), mild ISR <30%. LCx - mid 85% focal. RCA - nondominant, mild irregularities.

Intervention: Successful PCI of mid LCx with 3.0 x 18 mm drug-eluting stent. 0% residual, TIMI 3.

Complications: none. Biomarkers post: not elevated.

Plan: clopidogrel 75 daily x 6-12 mo with apixaban, stop ASA after 30 days.

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Pre-admission plan for device implant

EP PRE-PROCEDURE NOTE

10/02/2026 Okafor-Reyes, Gwendolyn MBR-57731

Surgeon: Hollis Varga, MD (EP)

Procedure: dual-chamber ICD (CPT 33249). Elective inpatient admission 10/15/2026.

Indication: ICM, LVEF 28% by TTE, NYHA III despite >3 yrs GDMT. SDM completed (see device clinic note).

No ACS in past year. Coronary hx per cardiology records.

Anticipated stay: 2 nights.

Supporting factors: apixaban held 48h pre-op w/ need for monitored restart and pocket checks while on clopidogrel; CKD 3a (eGFR 42) w/ planned post-procedure labs; PAF w/ risk of perioperative rapid AF on telemetry.

Exclusions reviewed: cognition intact, no terminal illness, AF rate controlled (HR 60s-70s), clinically stable. None present.

H. Varga MD

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